جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : صابرين يونس يونس احمد

المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** المساكن الاقتصاديه - حلوان

رقم البطاقة : 27601180103665 **رقم التليفون :** 01158980443 **تاريخ تسجيل الإستمارة :** 24-10-2026

العمر(تاريخ الميلاد) : 18-01-1976 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: صابرين يونس يونس احمد
National ID: 27601180103665

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

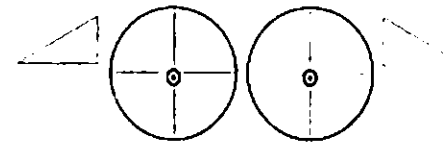
البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : صابرين يونس يونس احمد
2-ID number: 27601180103665

3- Medical number: 83068414
Sex: Female
Patient Age : 47 Y

MDT



Family history		Phone	01220461353
BMI	28	NRS Score	2
GROUP		marital status	MARRIED
Menopausal status	PRE	Co morbidity	DM ON INSULIN,,2 C.S
Date of MDT	30-Oct-2022	☐ REASON OF MDT	POST OPERATIVE

Clinical data

Breast Imaging :

Mets Workup :
CTCAP 3/2022--> PROMINENT RT INTERNAL MAMMARY LNS
BENIGN IN NATURE FOR FU
AB US 4/2022--> SIMPLE LEFT OVARIAN CYST FOR FU
BONE SCAN 3/2022-->FREE
SPONDYLO DEGENRATIVE CHANGES
ARE SEEN IN DORSO LUMBER VERTEBRAE

BX:	IHC:	ER:	PR:	Her 2 Neu :	Ki67 :
RT IDC GII 4/2022		0/8	4/8	+3	20

T	N	M
4B	2	0

Sonomammography
CESM 3/2022
RT BREAST : PATHOLOGICALLY PROVEN NEOPLASTIC PROCESS WITH PATHOLOGICAL LEVEL I AND II AXLN

Recommendations:
POST OPERATIVE RT MRM POST NACT
NO RESIDUAL VIABLE TUMOR FREE MARGIN
FREE NIPPLE AND AREOLA FREE LN 0/13 PATHOLOGICAL COMPLETE REPONSE YPT0 YPNO

MDT OUTCOME & PLAN

FOR PHYSGO TO COMPLETE OE YEAR, RADIOTHERAPY, ZOLADEX+AI. FOR FSH, LH, E2

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	

Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: صابرين يونس يونس	
MR. : 400	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

Breast cancer on femara+ zoladex for femara 3 months + zoladex.

FEMARA + SUPP >> 3M ZOLADEX TODAY C\O BONY ACHES >>> PT REFUSED RECIEVING ZOMETA CT CAP >> R.

50 Y/O F, Rt breast ca, cT4bN2M0, IDC, G2. Initial outside IHC: ER−, PR−, HER2 2+ with amplification. Repeat hormonal biopsy: ER−, PR 4/8, HER2 3+. Received NAC: Taxol + Carboplatin ×18 wks + Herceptin/Perjeta (HP) ×7 cycles. Underwent Rt MRM in 10/2022 → pCR (ypT0N0). Completed 1 yr anti-HER2 therapy (Herceptin/Perjeta) and adjuvant RT. Received Faslodex ×1 yr. Started Femara 11/2022 and Zoladex 01/2023, continued since then. CT CAP 5/2026 >>> Free of metastatic zoladex + femara for 3 months.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **Ali Adel**

يعتمد ,

Patient Name	: الأستاذه / صابرين يونس احمد	Request No.	: 13003862
Patient No.	: 27601180103665	Request Date	: 28/03/2022
Path No.	: SR-235/22	Verification Date	: 05/04/2022
Sex	: Female	Financial Category	: ورام دار السلام - مستشفى هرمل
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Ali mostafa

Pathology Report

Clinical Data:
Rt breast cancer.

Nature of the specimen:
Slide revision.

Gross Examination:
5 slides and one paraffin block labelled by (257/22) were received.

Microscopic examination:
Slides submitted showed single core of breast tissue with poorly processed scanty groups of atypical ductal cells. Immunostained slides showed suboptimal quality, unfit for reporting.

Diagnosis:
Rt breast, slide revision:
Poorly processed breast tissue.
Rebiopsy is recommended to confirm malignancy.

Prof.Dr.Eman Gouda

Verified by
Dr.Marwa mahmoud

Room No. (4) First Floor Building No. 867 N.la Corniche Nile El Qualaa Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / صابرين يونس احمد	Request No.	: 13006284
Patient No.	: 27601180103665	Request Date	: 09/04/2022
Path No.	: 963/2022	Verification Date	: 21/04/2022
Sex	: Female	Financial Category	: اورام دار السلام - مستشفى هرمل
Age	: 46 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass

Nature of the specimen:
Tru-cut biopsy

Gross Examination:
Five greyish white and hemorrhagic tissue cores up to 1cm . Totally submitted.

- Immunonistochemical technique: Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), &Ki67 (30-9)

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII(3+2+1)
- Biomarkers

Examination of ER stained slide show no nuclear staining in tumor cells.
Examination of PR stained slide show moderate nuclear staining in 10% of the tumor cells.
Examination of HER2 stained slide show strong staining in >10% of tumor cells with complete membranous staining.
Examination of Ki67 stained slide show nuclear staining in 20% of tumor cells.

Diagnosis:
Right breast mass ,Tru-cut biopsy :
Invasive duct carcinoma grade II
- ER: Negative (score 0/8).
- PR: Positive (score 4/8).

Verified by :
dr.menna atef

عمارة رقم (4) - الدور الأول - مبنى رقم (387) - شارع النيل - ميدان كابل - مصر القديمة - القاهرة - مصر
House No. (4) - First Floor-Building No. 387- Nile Corniche - Misr El Qadema- Cairo. Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / صبرين يونس احمد	Request No.	13006284
Patient No.	27601180103665	Request Date	09/04/2022
Path No.	963/2022	Verification Date	21/04/2022
Sex	Female	Financial Category	اورام دار السلام - مستشفى هرمل
Age	46 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- HER-2: Positive (score 3+)
- Ki67: Positive in 20% of tumor cells.

Prof.Dr.Gamal Abdelaty
Dr.Israa Elgendy

Verified by :
dr.menna atef

غرفة رقم (4) - الدور الأول - عمارة رقم (957) - كورنيش النيل - ميت عقبة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 957- Nile Corniche - Mist El Qadema- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

atient Name	: الأستاذه / صابرين يونس احمد	Request No.	: 13009144
Patient No.	: 27601180103665	Request Date	: 09/10/2022
Path No.	: 2979/22	Verification Date	: 16/10/2022
Sex	: Female	Financial Category	: مستشفى هرمل - ورام دار السلام
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Ali mostafa

Pathology Report

Clinical Data: Right breast cancer (post NACT) .
Nature of the specimen: Right MRM
Gross Examination:
• Procedure: Right MRM
measured 18x13x5 cm with covering skin ellipse measured 17x10cm & attached axilla 9x6 cm .
• Tumor extent : A greyish white tumour bed site
• Size: 5x5x1.5 cm .
• Skin involvement: Nipple & areola were unremarkable grossly .
• Distance from margins: 1 cm away from deep margin , 1.5 cm away from the skin .
• Lymph nodes number and size: 13 L.Ns up to 2.5x1 cm .
Microscopic examination:
• Histological type: No residual viable tumor tissue detected
• Ductal carcinoma in situ: No
• Skin involvement: Free nipple and areola
• Lymph node involvement: Free L.Ns (0/13)
• Resection Margins: Free margins
• Histologic response to therapy: Pathologic Complete response
Diagnosis:
Right MRM (post NACT) :
No residual viable tumor tissue detected.
• Free margins
• Free nipple and areola
• Free L.Ns (0/13)
• Pathologic Complete Response
• Stage : ypT0 , ypN0

Prof. Dr. Lubna Omar
Dr.Samah Sayed

Verified by :
saamah sayed

Room No. (4) First Floor Building No. 987 Nile Corniche Nasr El Qadema Cairo. Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

صابرين يونس

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

ة الصـــحة والسكـان
المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sabrin Yonis Yonis Ahmed	Date : 23/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 1200/2022	Pat. ID: 16762

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Report:

- Right MRM:
  - Operative bed shows no mass or collection
  - No regional lymphadenopathy.
- Left Breast:
  - ACR C with adenosis pattern
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Post right MRM status showing no evidence of recurrence (BIRADS 2)
- Left breast : fibroadenosis (BIRADS 2)

Signature Dr. Nehal Hamdy

23/10/2024 12:39 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name : Sabreen Younes Younes Ahmed
Branch : Maadi
Ref.Physician Prof : Medical Committe

Date : 29-06-2025 09:45
Medical No. : 138655
Ins.Company : التامين الصحي

^{99m}Tc-MDP W.B. Bone Scan

History:

- 49-year-old female patient with history of breast neoplasm.

Technique:

- *Whole body images were acquired 3 hours after injection of 740 MBq of the tracer.*

Findings:

- No features of osseous metastases detected.
- Mild degenerative changes of the spine

OPINION:

- **No scintigraphic evidence of osseous metastases.**

Much obliged,
Dr. Alaa Nagi, MD
Nuclear Medicine

[redacted]

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Women's Health Outreach Program

Name: Sabrine Younis Younis
Date: 03.03.2022

Contrast Enhanced Full field Digital Mammography

Clinical summary:
Pathologically proven right infiltrative neoplastic process G II

Technique
Pre & post contrast Mediolateral oblique and CC views
Targeted High resolution ultrasound

Detailed Findings

Left breast

- Heterogeneously dense breast Acr c
- No suspicious calcification or speculated mass lesions
- **Contrast enhanced images showed:** Nodular background enhancement is seen. no mass or non-mass enhancement
- Normal skin/nipple/areolar complex.
- No pathological axillary LNs.

Right breast

- Heterogeneously dense breast Acr c
- Central and UOQ irregular dense lesion as well as edema pattern is seen.
  - **Contrast enhanced images demonstrated:** Nodular background enhancement is seen. Central and UOQ intense heterogeneously enhancing mass lesion is seen measuring 5.9 x 5.5 x 8.2 cm, 2 cm from the nipple. No other enhancing lesions.
  - Level I & II pathological axillary lymph nodes the largest measuring 2.9 x 1 cm.

Conclusion:

- Right breast: pathologically proven neoplastic process with pathological level I & II axillary nodes as above described (BIRADS 6).

-Left breast: normal study (BIRADS 1)

Best regards,
Heba Azzam, MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Nuclear Medicine Unit ***وحدة الطب النووى***

Name :Sabreen Younes ***Date: 29-3-2022***

^{99m}Tc-MDP DUAL PHASE BONE SCAN

Brief History: female patient has left cancer breast for investigation.

Desired Information: skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP in the following manner:

Report: -

- *No hot spot that account for osseous deposits could be detected allover the rest of the surveyed skeleton.*
- *Spondylo-degenerative changes are seen in dorso-lumbar vertebrae.*

Conclusion: -

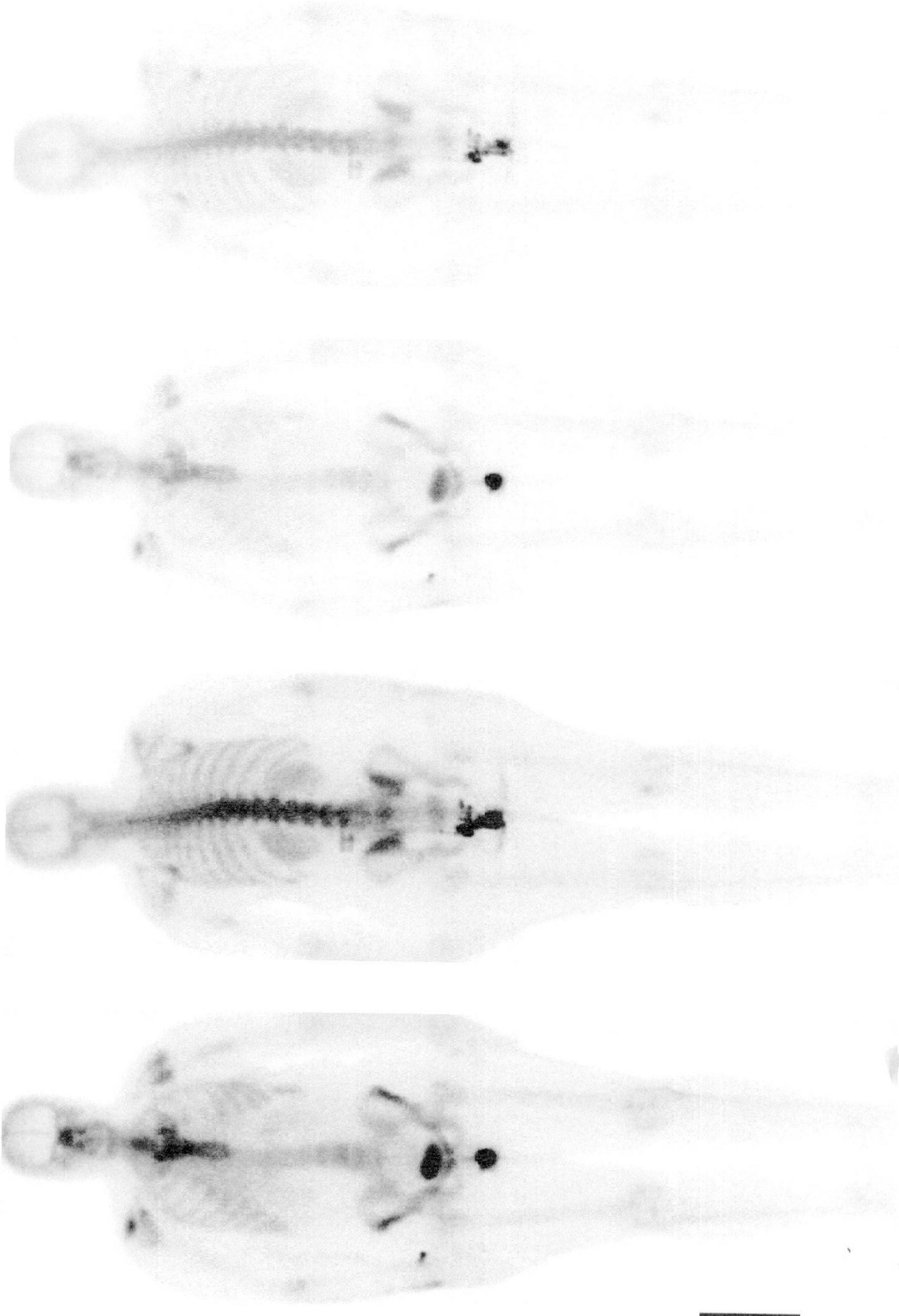
- *The surveyed skeleton is free of osseous deposits.*
- *Spondylo-degenerative changes are seen in dorso-lumbar vertebrae.*

Thanks for reference

Nuclear Medicine Unit

Prof. Huda Fathy, MD

Bone Scan
[[DOCTYPE:BONE_SCAN]]



PET Scan

[[DOCTYPE:PET_SCAN]]

- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

- No suspicious lytic or sclerotic lesions.

OPINION:

- ***Right breast malignant looking mass lesion with metastatic ipsilateral axillary lymph nodes as described.***
- ***Prominent right internal mammary lymph node.***
- ***Middle lung lobe pulmonary nodule likely benign in nature for follow up***
- ***No definite hepatic metastatic deposits.***
- ***Left adnexal cystic lesion for local US assessment.***

Dr .Mohammed Mohsen, MSc *Dr Ola Magdy, MD*

PET Scan
[[DOCTYPE:PET_SCAN]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: *S Younis*	***Date of birth:*** *18/01/1976*
Arabic Name: صابرين يونس	***Patient Gender:*** *F*
Patient RIS ID: *339816* ***Patient HIS ID:*** *123021944*	***Exam Date:*** *17/10/2023*
Written by: Dr. Hoda Fathy	**Approved by:** Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (16/10/2023)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the scoliotic dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged

Rawan Sherra

Nahla Mamoun,MSc

Bone Scan
[[DOCTYPE:BONE_SCAN]]

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Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sabrin Yonis Yonis Ahmed	Date : 16/11/2024
Exam : C.T CHEST	Age : 49 Year(s)
Medical ID: 1200/2022	Pat. ID: 16762

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

***As compared to 27/9/2022, The current revealed:**

- Right MRM showing no operative bed cysts,collections,or masses.
- Left breat dense parenchyma, for SM correlation.
- Still noted bilateral apical pleuropulmonary scarring, Likely old granulomatous sequel.
- Right upper lung lobe anterior and lateral segments subpleural faint reticulations and thin atelectatic bands ,Likely post radiation sequaie.
- ***Newly seen*** lingular inferior segment subpleural tiny puylmonary ground glass nodule.
- ***Otherwise***,Clear both lung fields,showing no sizable ssupicious pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Few prominent hilar and mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is seen mildldy to mopderately enlarged with iffuse homogenous low parenchymal CT texture. No definte focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic fccal lesions.

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

صحة والسكان
لطبية المتخصصة
ورام دار السلام
رة الاشعة

Patient Name: Sabrin Yonis Yonis Ahmed	Date : 16/11/2024
Exam : C.T CHEST	Age : 49 Year(s)
Medical ID: 1200/2022	Pat. ID: 16762

Opinion:

- **Newly seen lingular subpleural tiny ground glass nodule, likely inflamatory rather than metastatic for close follow up.**
- **Otherwise, Unremarkable CT scan of the chest, abdomen and pelvis.**
- **No evidence of pulmonary, hepatic or osseous metastatic deposits.**

Dr.Basma Mohamed.d

Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]